

Medical Interview
History



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Hawler Medical University
College of Medicine

Starting the consultation:

- **Establish initial rapport** (greet, introduce yourself and your role, obtain permission, demonstrate respect and interest, Pay attention to comfort and neatness, and most importantly, to safety).
- **Gain trust** (wash hands, listen, eye contact, clear instructions/explanations)

Identification data

- Name:
- Age:
- Sex:
- Religion:
- Marital state:
- Duration of marriage
- Occupation:
- Address:
- Date of admission:
- GPA
- LMP
- Gestational age
- List of patient`s problems.

PC (presenting complaint) or CC (chief complaint): what is the problem?

Could you tell me the main problem that led you to come to the clinic today? Invite the patient to tell you the story of the chief complaint by asking an open question

Showing Empathy

. Being empathic is a powerful way to build your relationship with patients.

. Empathy is the ability to identify with and understand patients' experiences, thoughts, and feelings.

. validate the patient's concerns. Patient's ideas and concerns

History of present illness (Gathering information):

During information Gathering:

Understand the Patient's Perspectives:

- Patient's expectation
- How each problem affects the patient's life
- Encourage expression of the patient's feeling

Use Cone system: Open questions probing questions and or closed questions): The following history illustrates the mix of question styles needed to elucidate a clear story:

- When did you first feel unwell, and what did you feel? (Open questioning)
- Well, I've been getting this funny feeling in my chest over the last few months. It's been getting worse and worse but it was really awful this morning. My husband called 999. The ambulance came and the nurse said I was having a heart attack. It was really scary.
- When you say a 'funny feeling', can you tell me more about what it felt like? (Open questioning, steering away from events and opinions back to symptoms):
- Well, it was here, across my chest. It was sort of tight, like something heavy sitting on my chest.
- How long was the tightness there? (Closed): About an hour altogether.
- So, you felt a tightness in your chest this morning that went on for about an hour and you also felt it in your neck? (Reflection): Yes, that's right.
- Did you feel anything else at the same time? (Open, not overlooking secondary symptoms): I felt a bit sick and sweaty.

❖ For each symptom ask about ***site, character, severity, radiation, association, onset (rapid or slow), duration, coarse, frequency, and pattern (day or night)} exacerbating & relieving factors.***

- ➢ Effect of disease on work (occupation) or on daily life like bathing, dressing, shopping & sleeping (awake her from sleep). ➢ Enquire about management to date.
- ➢ Attend to flow in logic consequence
- ➢ Attend to time
- ➢ Summarize
- ➢ Signpost next session

Characteristics of pain (SOCRATES)

1- Site

- Somatic pain is often well localized, e.g. sprained ankle
- Visceral pain is more diffuse, e.g. angina pectoris

2- Onset

- Speed of onset and any associated circumstances

3- Character

- Described by adjectives, e.g. sharp/dull, burning/tingling, boring/stabbing, crushing/tugging, preferably using the patient's own description rather than offering suggestions

4- Radiation

- Through local extension
- Referred by a shared neuronal pathway to a distant unaffected site, e.g. diaphragmatic pain at the shoulder tip via the phrenic nerve (C3, C4) ➢ Associated symptoms
- Visual aura accompanying migraine with aura
- Numbness in the leg with back pain suggesting nerve root irritation **5- Timing (duration, course, pattern)**
- Since onset
- Episodic or continuous
- If episodic, the duration and frequency of attacks
- If continuous, any changes in severity

6- Exacerbating and relieving factors

- Circumstances in which pain is provoked or exacerbated, e.g. food
- Specific activities or postures, and any avoidance measures that have been taken to prevent the onset.
- Effects of specific activities or postures, including effects of medication and alternative medical approaches

7- Severity

- Difficult to assess, as so subjective
- Sometimes helpful to compare with other common pains, e.g. toothache

- Variation by day or night, during the week or month, e.g. relating to the menstrual cycle.

History of Relevant system

Past history: PMH (Past Medical / Surgical History):

Current (active) and inactive medical conditions. Any history of previous hospital admissions, or operations, If so, when (year)? why? where (hospital?)

- **Childhood illnesses:** Measles, rubella, mumps, whooping cough, chicken pox, rheumatic fever, polio, scarlet fever, any childhood illnesses.
- **Adult illnesses:** DM, HTN, IHD, asthma, HIV, TB, stroke, fits, Cancer, hypercholesterolemia, renal disease, thyroid disease, others.
- **Past hospitalizations** :... ....(date diagnosis & treatment).
- **History of blood transfusions**:... .... (date & purpose).
- **History of any allergy** :..... .....(date & name).
- **Drug history**:..... .....(dose, route & duration).
- **Past surgical history** :.....(date & purpose).
- **Psychiatric history** :.....(dates diagnosis & treatment).
- **Gynecological history** :.....(menses & VB).
- **Past obstetric history**:.....G P A D
- **Immunization history** .....(date & name).

Family history:

Family history of diseases: DM, HTN, IHD, asthma, HIV, TB, stroke, fits, Cancer, hypercholesterolemia, renal disease, thyroid disease, and others.

Social history:

1) **Upbringing**:
2) **Occupations:** ..... (current, previous or unemployed).
3) **Marital state:** ..... (quality or any problem, sexual history).
4) **Travel history :**.....(drug or vaccination prophylaxis).

5) **Diet :**.....(high in CHO or fat, or drinking unpasteurized milk). 6) **Animal contacts: .....** cat, pigeon or cattle or others. 7) **Tobacco**: ..... Pack-year = no. of pack daily x years of smoking.

8) **Drug addiction**:
9) **Unit of alcohol**: (1U= 10 ml of pure alcohol), (x% proof = x U of alcohol **Alcohol:**/litter).

Review of Systems (ROS):

Cardiovascular system:

- Shortness of breath(dyspnea)
- Shortness of breath on lying down (Orthopnea)
- chest pain(on exertion, at rest)
- Tiredness on exertion
- Palpitation
- Ankle swelling
- Blue discoloration (Cyanosis)
- Intermittent claudication (leg cramps or leg pain on walking)

Respiratory system:

- Shortness of breath(dyspnoea)
- Cough
- Wheeze
- Sputum production (color, amount)
- Hemoptysis
- Hoarseness
- Chest pain

Gastrointestinal system:

- Dysphagia
- Odynophagia
- Nausea
- Vomiting

- Regurgitation ➢ Hematemesis
- Indigestion
- Heart burn
- Abdominal pain or distention
- Diarrhea
- Constipation
- Melena
- Rectal bleeding
- Yellow discoloration of skin and sclera (jaundice)

Genitourinary system:

- Frequency
- Dysuria
- Urgency
- Nocturia
- Hematuria
- Incontinence
- Urethral discharge
- Loin pain
- Suprapubic pain

CNS:

- Headache
- Dizziness
- Vertigo
- Faints
- fits
- Falls
- Numbness
- Tremor
- Speech disorders
- Loss of memory
- Loss or disturbance of vision (loss, blurring or double vision)

- Hearing (loss or tinnitus, hallucinations)
- Smell and taste disturbance
- Gait disturbance
- Gait disturbance

Musculoskeletal: ➢

Joint pain

- Joint stiffness
- Joint swelling
- Muscle pain
- Bone pain
- Limitation of joint movement
- Gait disturbance

Endocrine:

- Heat or cold intolerance
- Change in sweating
- Polydipsia
- Polyuria
- Polyphagia
- Prominence of eyes

Blood:

- Anemia
- Epistaxis
- gum bleeding
- easy bruising
- purpura

Skin:

- Rashes
- Itching
- Change in mole

Breast:

- Lumps
- Discharge
- Pain (mastalgia)

Building Relationship

- Use appropriate verbal and non-verbal behavior throughout the interview
- Develop Rapport
- Involve the patient

Management Explanation and Planning

Closing the interview

- Give any provisional information
- Check the patient's understanding and acceptance
- Encourage the patient to tell and discuss anything else you would like to talk about
- Summarize session
- Contract with the patient for the next session

Note:

When you are dealing with patients, always consider your:

- **A: attitude –** How would I feel in this patient's situation?
- **B: behavior –** Always treat patients with kindness and respect.
- **C: compassion –** Recognize the human story that accompanies each illness.
- **D: dialogue –** Listen to and acknowledge the patient.